



Nexus Vascular Imaging

Vascular Ultrasound Report
22 Bay Road Sandringham
admin@nexusvascularimaging.com

Patient Name: Julie Houston

Date of Birth: 14/12/1962

Report Date: 24/07/2026

Study: Bilateral Lower Limb Venous Ultrasound

Accession: WS-1388

Referring Doctor: Dr. Judy Wang

UR Number: UR 100092

Exam Date: 24/07/2026

Sonographer: Sam Johnson AMS 12704

Clinical Indication

Bilateral leg pain. Right leg visible varicose veins.

Findings

RIGHT:

Normal phasic flow in the common femoral vein suggests no ilio caval obstruction.

Segmental incompetence of the thigh and calf great saphenous vein. The great saphenous vein is straight and subfascial for the length of the thigh.

Incompetent anterior accessory saphenous vein and anterior thigh circumflex vein, sourced via the superficial epigastric vein. The anterior accessory saphenous vein is straight for 22 cm, becoming extrafascial distally.

Incompetent distal short saphenous vein.

No deep or perforator incompetence.

LEFT:

Normal phasic flow in the common femoral vein suggests no ilio caval obstruction.

Segmental incompetence of the thigh and calf great saphenous vein with reflux to a medial thigh tributary. The great saphenous vein is straight and subfascial for the length of the thigh.

Incompetent distal short saphenous vein.

No deep system incompetence.

Incompetent medial thigh and lateral calf perforators.

Impression

Right: Great saphenous vein and anterior accessory saphenous vein incompetence suitable for endovenous intervention. Distal short saphenous vein incompetence.

Left: Great saphenous vein incompetence suitable for endovenous intervention. Distal short saphenous vein and perforator incompetence.

Mr Sam Farah MBBS(Hons) MMed FRACS
Vascular Surgeon

To: Dr. Judy Wang

Copies to: reception@hsmc.com.au

Electronically signed 24 July 2026

BILATERAL LOWER LIMB VENOUS ULTRASOUND

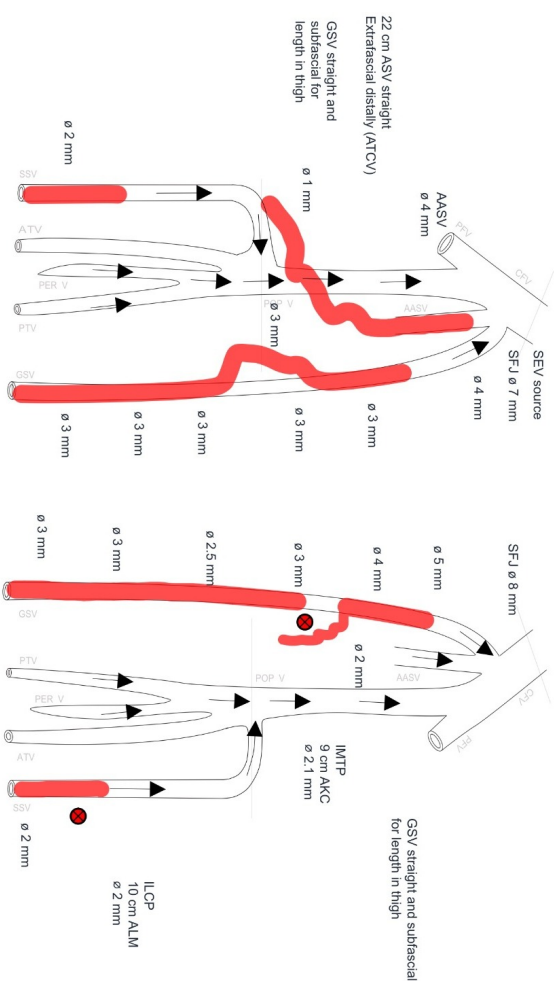
Reflux

Thrombus

Incompetent perforator

R

L



COMMENTS: